BASELINE → TREATMENT → FOLLOW-UP

Required safety checks and your individual disease-assessment plan

The team supplies actual dates and resolves any source ambiguity. A reference period does not replace a prescription, a local monitoring plan or an individual treatment decision.

Check	Before starting	During treatment	Between visits	Response / follow-up	After a hold or stop
Bloods and organ safety	Full blood count, kidney and liver tests, glucose and thyroid tests; BRAF tumour testing and review of autoimmune conditions, transplantation, prior immune reactions and current medicines.	Full blood count, kidney/liver tests, glucose and thyroid tests each treatment cycle. Cortisol and other tests are added when clinically indicated. Every visit also reviews bowel, breathing, skin, endocrine, neurological and other symptoms; a normal blood test does not exclude immune toxicity.	Contact the team for new symptoms; extra tests may be needed between scheduled checks.	Safety bloods do not by themselves show whether cancer is responding.	Continue to report possible immune effects for at least five months after the last dose, as specified by NCCP, and for longer whenever your team advises. Keep any steroid taper, hormone-replacement plan and relevant follow-up tests. Response imaging follows your individual plan.
Cancer, skin and daily function	Record diagnosis, stage, current disease sites and the baseline assessment.	Keep the source-specific skin and clinical reviews and the individually agreed response assessments.	Record new lesions, lumps, wound changes, pain and effects on usual activities.	Write the next examination or scan date and the responsible team; no universal scan or tumour-marker interval is assumed.	Follow-up depends on stage, treatment and symptoms. Your team records the skin/lymph-node examination plan, whether body or brain imaging is needed, and which service owns each appointment. A general patient booklet does not set a universal scan interval for every patient.
Medicines and continuing precautions	Review all medicines and the pregnancy/fertility plan.	Review treatment taken or received, holds, adverse effects and supportive medicines.	Keep the actual urgent contacts and do not restart a held medicine yourself.	Bring the medicine list and treatment summary to other services.	Discuss fertility and pregnancy precautions before starting. Immunotherapy may harm a pregnancy. Use the contraception plan and after-treatment duration specified for the medicines you receive; ask about breastfeeding restrictions. Tell the team promptly about possible pregnancy. Do not stop necessary hormone replacement or prescribed steroids without advice.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00431 v11: current regimen source · NCCN Guidelines for Patients: Melanoma, 2026 - supplied education reference

THE WHOLE TREATMENT JOURNEY

Nivolumab and ipilimumab for advanced melanoma: treatment and checks

NCCP 00431a: advanced unresectable or metastatic melanoma in adults. This patient guide is the melanoma context, not the separate liver-cancer indication.

The combination phase gives nivolumab followed by ipilimumab by IV infusion every three weeks for up to four doses. If appropriate, nivolumab alone then continues on the team-selected two-weekly or four-weekly schedule, by the prescribed IV or subcutaneous route. After the last combination dose, the first two-weekly maintenance dose starts three weeks later; the first four-weekly maintenance dose starts six weeks later in this melanoma schedule. Treatment continues until progression or unacceptable toxicity, subject to review.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	During treatment	Between visits	Response / follow-up	After a hold or stop
Bloods and organ safety NCCP 00431	Full blood count, kidney and liver tests, glucose and thyroid tests; BRAF tumour testing and review of autoimmune conditions, transplantation, prior immune reactions and current medicines.	Full blood count, kidney/liver tests, glucose and thyroid tests each treatment cycle. Cortisol and other tests are added when clinically indicated. Every visit also reviews bowel, breathing, skin, endocrine, neurological and other symptoms; a normal blood test does not exclude immune toxicity.	Contact the team for new symptoms; extra tests may be needed between scheduled checks.	Safety bloods do not by themselves show whether cancer is responding.	Continue to report possible immune effects for at least five months after the last dose, as specified by NCCP, and for longer whenever your team advises. Keep any steroid taper, hormone-replacement plan and relevant follow-up tests. Response imaging follows your individual plan.
Cancer, skin and daily function NCCP 00431	Record diagnosis, stage, current disease sites and the baseline assessment.	Keep the source-specific skin and clinical reviews and the individually agreed response assessments.	Record new lesions, lumps, wound changes, pain and effects on usual activities.	Write the next examination or scan date and the responsible team; no universal scan or tumour-marker interval is assumed.	Follow-up depends on stage, treatment and symptoms. Your team records the skin/lymph-node examination plan, whether body or brain imaging is needed, and which service owns each appointment. A general patient booklet does not set a universal scan interval for every patient.

Care / check	Before starting	During treatment	Between visits	Response / follow-up	After a hold or stop
Medicines and continuing precautions NCCP 00431	Review all medicines and the pregnancy/fertility plan.	Review treatment taken or received, holds, adverse effects and supportive medicines.	Keep the actual urgent contacts and do not restart a held medicine yourself.	Bring the medicine list and treatment summary to other services.	Discuss fertility and pregnancy precautions before starting. Immunotherapy may harm a pregnancy. Use the contraception plan and after-treatment duration specified for the medicines you receive; ask about breastfeeding restrictions. Tell the team promptly about possible pregnancy. Do not stop necessary hormone replacement or prescribed steroids without advice.

What each check is for

1 Combination induction

Up to four visits

Nivolumab and ipilimumab help immune cells recognise and attack melanoma. The combination phase is followed by nivolumab alone when appropriate. The aim in this setting is to control advanced disease; benefit and toxicity are reviewed for the individual.

Ask / act: Confirm the current treatment phase, prescription, tests and support medicines.

Sources: NCCP 00431

2 Nivolumab alone

Maintenance

Nivolumab and ipilimumab help immune cells recognise and attack melanoma. The combination phase is followed by nivolumab alone when appropriate. The aim in this setting is to control advanced disease; benefit and toxicity are reviewed for the individual.

Ask / act: Confirm the current treatment phase, prescription, tests and support medicines.

Sources: NCCP 00431

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Sources checked 2026-11-06 · Draft source reconciliation; independent clinical, pharmacy and patient/PPI review pending · SACTCheck v0.84.40